

Prescription For Life

A study edition of the Practical Love recommendation of Prescription For Life by Dr. Richard Furman, MD, FACS, with explanations on why a long life is worth seeking and how to read a health book wisely.

A book is only a prescription. The taking of it is yours.

Practical Love Publications

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Contents

About This Study Edition	3
Why a Ministry of Love Recommends a Health Book	4
What a Recommendation Is, and How to Weigh One	6
Point One: Based on Documented Medical Research	7
Point Two: Three Simple Strategies to Live Younger and Longer	9
Point Three: Six Foods You Should Never Eat Again	10
Point Four: Why Lack of Exercise Is Killing You	12
Point Five: Decrease the Risk of Heart Attack, Stroke and Dementia	14
The Postscript: A Word to Every Nigerian	16
As Powerful as The Purpose Driven Life	17
The Body You Were Given	19
Reading the Book Well: A Plan	20
A Prayer and a Commitment	22

About This Study Edition

FROM THE ORIGINAL LEAFLET

PRESCRIPTION FOR LIFE

By Dr. Richard Furman, MD, FACS

1. Based on documented medical research.
2. Three simple strategies to live younger and longer.
3. Six (6) foods you should NEVER eat again.
4. Why lack of exercise is killing you.
5. Decrease the risk of heart attack, stroke, and dementia.

P.S. This book is a must read for every Nigerian in order to increase our average life-span. This book is as powerful as *The Purpose Driven Life* by Rick Warren, with over sixty million copies sold worldwide. I have prayerfully read PDL over ten times, each time gaining a new perspective.

The original leaflet printed above is not a book. It is a recommendation of a book. It fits on a single page, it names five things the reader will find inside *Prescription For Life* by Dr. Richard Furman, and then it adds a short postscript urging every Nigerian to read it. That is the whole of it. A person could read the leaflet in under a minute and put it down without ever asking why a doctor of the gospel would take the trouble to print and hand out a recommendation for another man's book.

This study edition asks that question and works through the answer slowly. It takes the five points one at a time, explains what each one is claiming and why it matters, sets the whole recommendation inside what the Bible says about the body God has given us, and ends with a plan for reading the book well and questions to carry into prayer. The aim is not to replace the book but to make the reader ready for it.

What this edition is and is not

Everything printed in a box marked "From the original leaflet" is the wording of the recommendation as it was handed out, with spelling and punctuation lightly corrected. Everything else in this booklet — the explanations, the background, the practical steps and the questions — is study notes prepared by Practical Love Publications. These notes are not the original writer's own words, and they are not the words of Dr. Furman either.

This edition deliberately does not summarise the contents of *Prescription For Life*. It does not list the six foods, it does not name the three strategies, and it does not repeat the research the book cites. There are two reasons for that. The first is honesty: the leaflet itself does not tell us what those six foods are, and these notes will not invent an answer and put it in another man's mouth. The second is respect: *Prescription For Life* is Dr. Furman's own work, and the right way to receive it is to obtain the book and

read it, not to swallow somebody else's digest of it.

These notes prepare you to read the book. They are not a substitute for it.

An important word about health advice

This booklet is a Christian study aid, not a medical consultation. Nothing in these notes should be taken as a diagnosis, a treatment plan, or a reason to start, stop or change any medicine. If you are living with high blood pressure, diabetes, heart disease, kidney disease, or any condition at all, and certainly if you are pregnant or caring for a child, take every question raised here to a qualified doctor who can examine you and who knows your history. The general facts stated in these notes are the ordinary, settled teaching of public health; the application of them to your own body belongs to you and your physician.

How to use this study

- **Personal reading.** Read one section at a time, with your Bible open and a pen in hand, and write down the single change you are willing to make before you turn to the next section.
- **With your family.** Read the boxed leaflet text aloud, then one section, and let each member of the household answer the closing question in their own words. Health habits are learned at the family table long before they are learned from a book.
- **In a house fellowship or church group.** Let one person read the boxed text, let others read the Scriptures, and use the questions at the end of each section for discussion. Set a date some weeks ahead when the group will meet again and report what actually changed.
- **As a leader or pastor.** Each section can stand on its own as the basis of a short talk to a men's meeting, a women's fellowship, a youth group or a workers' retreat.

Scripture quotations in these notes are from the King James Version (KJV). You are encouraged to look up each passage in your own Bible, to read the verses around it, and to satisfy yourself that the explanation offered here is faithful to what is written.

Why a Ministry of Love Recommends a Health Book

The first thing a careful reader notices about this leaflet is that it seems out of place. Practical Love Publications exists to press the commandment of love upon homes, churches and a nation. Its other leaflets speak about loving God, loving one's neighbour, raising children in the fear of the Lord, and the corruption that follows when love grows cold. Then, in the middle of that shelf, stands a single page recommending a book by an American surgeon about food and exercise. Why?

The answer is that love is practical or it is nothing. A love that speaks warmly and leaves the beloved to die early of a preventable disease is not the love the Bible commands. The apostle John puts the test very plainly:

But whoso hath this world's good, and seeth his brother have need, and shutteth up his bowels of compassion from him, how dwelleth the love of God in him? My little children, let us not love in word, neither in tongue; but in deed and in truth.

1 John 3:17-18 (KJV)

Knowledge is one of this world's goods. If a man knows what shortens life and keeps quiet while his brother walks into it, he has shut up his compassion just as surely as the man who withholds money. Handing somebody a leaflet that says *read this book* is love in deed, not merely in word. It costs the giver something — the price of printing, the awkwardness of pressing a point — and it offers the receiver something they can actually use.

The gospel has always come with healing beside it

There is a second reason, older than any leaflet. The ministry of the Lord Jesus was never a ministry of words only. The Gospel writers keep joining two things together in the same breath:

And Jesus went about all Galilee, teaching in their synagogues, and preaching the gospel of the kingdom, and healing all manner of sickness and all manner of disease among the people.

Matthew 4:23 (KJV)

Teaching, preaching, healing. Three strands in one cord. The church that carries only the first two is carrying a cord that has been cut. Down the centuries the followers of Christ have built hospitals, dug wells, trained nurses and taught mothers how to feed infants, not as a distraction from the gospel but as part of its natural outworking. A leaflet about food and exercise stands in that long line.

Nigeria is named on purpose

The postscript does not say *this book is a must read for everybody*. It says it is a must read “for every Nigerian in order to increase our average life-span”. The word “our” is the hinge. This is not a foreigner lecturing from a distance; it is a Nigerian speaking to Nigerians about a Nigerian problem, using a tool that happens to have been written elsewhere. That is worth noticing, because a recommendation carries weight in proportion to how much the person making it shares your life.

Love that knows and says nothing is not love. It is only politeness.

For reflection

1. Who has ever told you something about your health that you did not want to hear? Did you receive it as love or as interference? How do you view it now?

2. Is there someone in your household or fellowship whose habits worry you, and whom you have avoided speaking to? What has held you back?
3. In what practical way did somebody love you *in deed and in truth* this past month?

What a Recommendation Is, and How to Weigh One

Before working through the five points, it is worth pausing on the form of the thing. A recommendation is a particular kind of speech. It is not a sermon, which teaches with authority. It is not a report, which sets out evidence for you to judge. A recommendation is a person saying: *I have been somewhere, and I think you should go too*. Its whole strength rests on who is speaking and how well they know both the book and you.

Two names stand behind this page

The first name is the author's: Dr. Richard Furman, MD, FACS. The letters matter. MD is the medical degree. FACS means Fellow of the American College of Surgeons, a recognised standing among surgeons in the United States. The leaflet is telling you, in shorthand, that the book was not written by an enthusiast with a theory but by a trained and credentialled surgeon.

The second name is the one that does not appear in the five points at all: the person who printed and handed out the leaflet. It is their judgement you are being asked to trust. The postscript is where that person steps forward and speaks for themselves — “I have prayerfully read PDL over ten times” — and that sentence tells you the kind of reader they are before it tells you anything about the book.

How to weigh any health book

Nigeria is not short of health advice. It is shouted from buses, printed on handbills, preached on the radio and forwarded endlessly on the phone. Much of it is harmless, some of it is helpful, and a portion of it is dangerous. The believer needs a way to sort it. Here are five questions that will serve for almost any book, broadcast or message that reaches you:

1. **Who wrote it, and what are they trained in?** Training is not everything, but a claim about the heart carries more weight from a heart surgeon than from a stranger with a microphone.
2. **What is the evidence?** Does the writer point to research that other people can check, or only to stories of people who got better?
3. **What is being sold?** A book that exists to move you towards habits you can practise free of charge is a different thing from a book that exists to move you towards a product the writer happens to sell.

4. **Is the promise modest or extravagant?** Honest health advice speaks of reducing risk. Dishonest advice speaks of curing everything.

5. **Does it agree with what my own doctor can verify?** A book cannot examine you. A doctor can.

Measured by those questions, the leaflet's own claims are notably modest. It does not promise healing. It does not promise long life as a guarantee. Point five says only "decrease the risk" — and that restraint is itself a mark of an honest book.

The simple believeth every word: but the prudent man looketh well to his going.

Proverbs 14:15 (KJV)

Scripture does not treat gullibility as a virtue. The prudent man looks well to his going — he examines the road before he walks it. Weighing a health claim carefully is not unbelief; it is obedience to this proverb.

For reflection

1. Think of the last piece of health advice that was forwarded to you. Run it through the five questions above. How does it stand?
2. Whose word do you trust most about your body, and why?
3. Have you ever passed on advice you had not checked? What might that have cost the person who received it?

Point One: Based on Documented Medical Research

FROM THE ORIGINAL LEAFLET

1. Based on documented medical research.

This is the first thing the leaflet says about the book, and its placement is deliberate. Before any promise is made, the reader is told what kind of ground the promises are standing on. The word to weigh is *documented*. It means that the claims in the book are not the author's private opinion or the accumulated hunches of a long career, but are tied to studies that were written down, published, and are open to anybody who wants to check them.

Why documentation matters so much

Everyone has heard of the uncle who smoked forty years and lived to ninety, and the neighbour who ate carefully and died at fifty. Those stories are true, and they prove nothing at all, because one life cannot

separate what a habit did from what would have happened anyway. Medical research exists to solve exactly that problem. By following large numbers of people over long periods and comparing groups, it can show what a habit does on average, across thousands of lives, which is something no single story can ever show.

This is why a book that says *the research shows* is making a stronger and more testable claim than a book that says *in my experience*. It is also why such a book is easier to hold to account: if the studies are named, a reader with time and access can go and look.

What research can and cannot tell you

- **It can tell you about risk, not about certainty.** Research says that a habit raises or lowers the chance of an outcome across a population. It cannot tell any individual what will happen to them.
- **It can be revised.** Honest science changes its mind when better evidence arrives. That is a strength, not a weakness, though it can be unsettling to a reader who wants a final word.
- **It is often reported badly.** A careful study can become a reckless headline in a single step. When you hear a startling health claim, ask where it came from before you act on it.
- **It does not answer questions of meaning.** No study can tell you why life is worth preserving, or what to do with the years you gain. For that you need a different book entirely.

Truth and the God of knowledge

Some believers grow uneasy at this point, as though trusting research were a rival to trusting God. Scripture does not set them against each other. The Lord is himself the God of knowledge, and honest investigation of the world he made is not a betrayal of faith but a form of attention to his works.

Talk no more so exceeding proudly; let not arrogancy come out of your mouth: for the LORD is a God of knowledge, and by him actions are weighed.

1 Samuel 2:3 (KJV)

If the Lord is a God of knowledge, then knowledge patiently gathered about the body he formed is not the enemy of faith. What Scripture condemns is not learning but arrogance — the man who has stopped weighing his actions because he is sure he already knows. That warning falls on the proud researcher and the proud believer alike.

Faith and evidence are not rivals. Pride is the enemy of both.

For reflection

1. When you last heard a health claim that surprised you, did you check where it came from?
2. Is there an area of your life where you have preferred a comforting story to a well-founded fact?

3. How would you explain to a friend the difference between *one person got better* and *this habit lowers risk*?

Point Two: Three Simple Strategies to Live Younger and Longer

FROM THE ORIGINAL LEAFLET

2. Three simple strategies to live younger and longer.

The second point is the book's central promise, and it contains two ideas that deserve to be taken apart: that the strategies are *simple*, and that they aim at living *younger* as well as longer. These notes will not name the three strategies, because the leaflet does not name them and it would be dishonest to guess on Dr. Furman's behalf. What can be examined is the shape of the promise.

The word “simple” is doing hard work

Simple does not mean easy. A thing can be perfectly simple to understand and still cost a great deal to do. Forgiving an enemy is simple to state and hard to perform. So it is with health: almost nobody in Nigeria needs to be told that fried food, sugar and a seated life are working against them. The information is not the scarce commodity. The scarce commodity is doing it, day after day, when nobody is watching and nothing appears to be changing.

A book that offers three strategies rather than thirty is making a bet that people will actually follow a short list. That bet is sound. The mind can hold three things. It can also defend three things when a friend asks why you are refusing a second helping.

Living younger, not merely longer

The phrase “live younger and longer” is carefully chosen. Length of life and quality of life are not the same thing, and a person can gain years that are not worth having. To live younger is to keep the capacity for ordinary life: to climb stairs, to carry a grandchild, to walk to church, to think clearly, to sleep, to work. That is what most people actually want when they say they want to live long.

Scripture thinks the same way. When it speaks well of old age it speaks of strength retained, not merely of years accumulated:

And Moses was an hundred and twenty years old when he died: his eye was not dim, nor his natural force abated.

Deuteronomy 34:7 (KJV)

The verse is not a promise that every believer will reach a hundred and twenty. It is a portrait of the kind of long life that is worth desiring: eyes that still see, strength that has not drained away. That is living younger and longer in the language of the Bible.

Why a short list beats a long resolution

1. **Three things can be remembered.** A plan you cannot recall on a busy Tuesday is not a plan.
2. **Three things can be measured.** You can honestly say at the end of a week whether you did them.
3. **Three things can be taught.** You can pass them to your spouse, your children and your fellowship without printing anything.
4. **Three things leave room for grace.** A person who fails at one of three can hold the other two. A person who fails at one of thirty usually abandons all thirty.

For reflection

1. If you had to name three changes that would most improve your own health, what would they be? Write them down before you read the book, then compare.
2. What is the difference, in your own family, between the relatives who have aged well and those who have not?
3. Is there a good thing you already know to do and have not begun? What would it take to begin this week?

Point Three: Six Foods You Should Never Eat Again

FROM THE ORIGINAL LEAFLET

3. Six (6) foods you should NEVER eat again.

This is the boldest line on the leaflet, and the capital letters are in the original. It is also the line most likely to make a reader reach for the book, which is presumably why it was placed in the middle of the list. Six is a small enough number to feel manageable. *Never again* is strong enough to feel serious.

Why these notes do not list the six

The leaflet does not say which six foods are meant, and these notes will not guess. To print a list here and let readers assume it came from Dr. Furman would be to put words in his mouth, and a study edition that begins by inventing its subject matter has failed before it starts. There is also a plain matter of right: *Prescription For Life* is his book, and its particular findings belong in it. The honest course is to say what

the leaflet says, explain why the claim is worth taking seriously, and send the reader to the source.

The leaflet names a promise, not a list. For the list, read the book.

What is not in doubt

While the book's own six remain the book's own, the general direction of public health teaching about food is not a secret and is not disputed among those who study it. The following is ordinary, settled guidance, offered here as background and not as the contents of Dr. Furman's list:

- Diets heavy in deep-fried food, especially oil reheated many times, are associated with higher risk of heart disease.
- Sugary soft drinks add a great deal of sugar without satisfying hunger, and are strongly linked to weight gain and to type 2 diabetes.
- A high salt intake raises blood pressure in many people, and high blood pressure is one of the chief drivers of stroke.
- Processed and preserved meats are linked to higher rates of certain diseases than fresh meat is.
- Vegetables, fruit, beans and whole grains are consistently associated with better outcomes across almost every category of disease studied.

None of that is new, and none of it is foreign to Nigerian food. The traditional table of this country — beans, vegetables, yam, plantain, fish, groundnut, fruit — is closer to what the research commends than the newer table of soft drinks, packaged snacks and repeatedly fried oil. In many homes the healthier diet is not a foreign import to be purchased but an older habit to be recovered.

The trouble with the word “never”

A rule that says *never again* has one great strength and one great danger. Its strength is that it removes the daily argument. The man who has decided never is not negotiating with himself at every meal, and that saves an enormous amount of strength. Its danger is despair: the person who breaks an absolute rule once often concludes the whole project has collapsed and abandons it entirely.

The believer has a particular resource here, because the gospel has a great deal to say about beginning again after failure. A single bad meal is not a verdict on your life, any more than a single bad day is a verdict on your walk with God.

For a just man falleth seven times, and riseth up again: but the wicked shall fall into mischief.

Proverbs 24:16 (KJV)

The mark of the righteous is not that they never fall. It is that they get up. Apply that to a change of diet and it becomes a practical rule: never use one failure as the excuse for the next.

Appetite and self-control

Scripture is not silent about food. It treats appetite as one of the areas in which the self is either governed or allowed to govern, and it does so without either despising food or worshipping it.

For many walk, of whom I have told you often, and now tell you even weeping, that they are the enemies of the cross of Christ: whose end is destruction, whose God is their belly, and whose glory is in their shame, who mind earthly things.

Philippians 3:18-19 (KJV)

Paul is not attacking dinner. He is describing people for whom appetite has become the final authority — whose belly is their God. That is a spiritual condition before it is a dietary one, and it is why a change of eating that is attempted with no change of heart so often lasts about three weeks.

At the same time, food is a gift, and Scripture refuses the opposite error of treating abstinence as holiness in itself. Paul warns Timothy against teachers “commanding to abstain from meats, which God hath created to be received with thanksgiving” (1 Timothy 4:3). The believer’s aim is neither indulgence nor superstition, but a table governed by gratitude and self-control.

For reflection

1. Which food would be hardest for you to give up entirely, and what does that difficulty tell you?
2. What did your grandparents eat that your household no longer eats? Was the change an improvement?
3. When you have failed at a resolution before, what usually happened on the day after the first failure?

Point Four: Why Lack of Exercise Is Killing You

FROM THE ORIGINAL LEAFLET

4. Why lack of exercise is killing you.

The fourth point is phrased more sharply than the others. It does not say that exercise is beneficial. It says that its absence is killing you — present tense, continuous, happening now. The leaflet is refusing to let inactivity be understood as a neutral state, a simple absence of something good. It is presenting it as an active cause of harm.

Why the sharp wording is fair

Most people file exercise under *good if you have time*, in the same drawer as learning a language. The research does not support that filing. A body that is never made to work loses capacity steadily and quietly: muscle declines, the heart and lungs lose conditioning, blood sugar is handled less well, weight

accumulates, bones weaken, sleep worsens and mood suffers. None of this announces itself. It is discovered years later at a clinic, or on a flight of stairs.

This is why the leaflet's wording is not exaggeration but accuracy. A slow harm is still a harm. Calling it by its right name is the only way to move it out of the drawer marked *someday*.

The shape of modern life works against the body

It is worth being fair to the reader here. Very few people choose inactivity. It is built into how most of us now live: long hours seated at work, long hours seated in traffic, errands done by vehicle that a previous generation walked, evenings spent before a screen, and in many towns nowhere safe or lit to walk in the first place. The problem is structural before it is personal.

That does not remove responsibility, but it does change the strategy. A person fighting their environment needs a plan that fits inside it rather than a plan that assumes it away. The question is not *how do I become an athlete* but *where, in the life I actually have, can the body be made to work?*

Movement that costs nothing

1. **Walk the parts of a journey you can walk.** Leave the vehicle earlier and finish on foot.
2. **Use the stairs whenever they exist and your knees allow.**
3. **Stand and move every hour of seated work,** even for two minutes. Set a reminder if you must.
4. **Make one walk a fixed appointment** — early morning or evening, the same time each day, with somebody who will notice if you do not come.
5. **Bring the family in.** A walk after the evening meal is exercise, conversation and family devotion rolled into one.
6. **Count the work you already do.** Sweeping, farming, carrying, washing and gardening are real physical work and should not be dismissed because they happened at home.

Notice that none of these require a gymnasium, equipment, special clothing or money. That matters, because a health plan that can only be followed by the comfortable is of no use to the majority of a nation.

Discipline is not a foreign idea to faith

But I keep under my body, and bring it into subjection: lest that by any means, when I have preached to others, I myself should be a castaway.

1 Corinthians 9:27 (KJV)

Paul is speaking of spiritual discipline, and he reaches for the picture of an athlete in training to explain it. The point stands in both directions. The believer who accepts that the soul requires daily discipline, and then treats the body as though it required none, has only half-understood the apostle's illustration.

Bodily training is not the goal of the Christian life; but a body deliberately neglected will in time limit the service you are able to offer.

Paul is careful to keep the order right elsewhere: “For bodily exercise profiteth little: but godliness is profitable unto all things” (1 Timothy 4:8). Little is not nothing. Godliness comes first, and exercise takes its proper place beneath it — worth doing, not worth worshipping.

Godliness first. But little is not nothing.

For reflection

1. How many minutes did your body actually work yesterday? Count honestly, including housework and walking.
2. What is the single realistic change to your week that would add the most movement?
3. Who could walk with you, so that the habit has a witness?

Point Five: Decrease the Risk of Heart Attack, Stroke and Dementia

FROM THE ORIGINAL LEAFLET

5. Decrease the risk of heart attack, stroke, and dementia.

The last of the five points is the most restrained, and its restraint is the best evidence of the book’s honesty. It does not say *prevent*. It does not say *cure*. It says *decrease the risk*. Anybody who has sat with a family after a sudden death knows how much that distinction matters, and a writer who keeps it is a writer worth reading.

Three diseases, largely one set of causes

Heart attack, stroke and dementia present very differently — a chest, a brain, a memory — but a great deal of what drives them overlaps. Broadly, and as general public health teaching rather than as any claim about this particular book, the same short list of factors appears behind all three:

- **High blood pressure**, which is the single most important cause of stroke and often produces no symptoms at all until damage is done.
- **Smoking**, which damages blood vessels throughout the body.
- **Diabetes and high blood sugar**, which injure vessels slowly and persistently.
- **Excess weight, poor diet and inactivity**, which drive the three factors above.

- **Untreated conditions**, because a disease that is known and managed is a very different thing from a disease that is ignored.

The practical consequence is encouraging: the same handful of changes works against all three at once. A person is not being asked to run three separate campaigns. They are being asked to run one.

The silent killer and the value of a simple check

High blood pressure deserves a paragraph of its own, because it is common in Nigeria, it is frequently undiagnosed, it usually causes no symptoms for years, and it is treatable. A great many strokes that devastate families were preceded by a number that nobody had ever measured.

If this study edition achieves nothing else, let it achieve this: **have your blood pressure checked**. It takes a few minutes, it is widely available at clinics and pharmacies, and it is the cheapest serious thing most adults can do for their health. If it is high, do not treat the reading as a curse or a rumour. Take it to a doctor and follow the treatment given, including on the days you feel perfectly well.

Get your blood pressure checked. It is the cheapest serious act of self-care available.

Risk, providence and the length of our days

Some readers will object that the length of life is in God's hand and that all this calculation of risk is unbelief. Scripture certainly teaches the first part:

Seeing his days are determined, the number of his months are with thee, thou hast appointed his bounds that he cannot pass.

Job 14:5 (KJV)

But Scripture nowhere draws from that truth the conclusion that means are pointless. The same Bible that says our bounds are appointed also commands a parapet around the roof so that no one falls from it (Deuteronomy 22:8), records Paul telling the centurion that the sailors must stay in the ship if the passengers are to be saved (Acts 27:31), and has Paul advising Timothy on what to drink for his stomach's sake (1 Timothy 5:23). God's sovereignty over the end never cancels our responsibility for the means.

The wise position is therefore neither anxiety nor fatalism. We do what is set before us to do, we take the checks and the treatment that are available, we eat and move as those who have been entrusted with something, and we leave the outcome where it has always been.

For reflection

1. When were you last checked for blood pressure and blood sugar? If you cannot remember, when will you go?

2. Has anyone in your family suffered a stroke, a heart attack or dementia? What did it cost the household?
3. Where, in your own thinking, does trust in God shade into an excuse for doing nothing?

The Postscript: A Word to Every Nigerian

FROM THE ORIGINAL LEAFLET

P.S. This book is a must read for every Nigerian in order to increase our average life-span.

The five points describe a book. The postscript is where the person handing out the leaflet stops describing and starts pleading. It is the only sentence on the page that names a country, and it is the reason the leaflet exists at all.

“Our” average life-span

Average life-span is a national measurement. It does not describe any individual; it describes a people. To speak of increasing *our* average is to say that health is not only a private matter of one man’s diet but a public matter of how a whole nation lives, eats, moves, works and is cared for.

That framing is important for a ministry concerned with love, because it makes health a matter of neighbourly obligation rather than personal preference. A national average rises when households change, when churches teach, when markets sell differently, when employers allow movement, and when clinics are within reach. Every one of those is a place where love for neighbour has practical work to do.

The premature deaths behind the number

Behind a low average sit particular funerals: men in their fifties, women who did not see their grandchildren, breadwinners who left school fees unpaid and widows without provision. A congregation that has buried several such members in a single year has felt this statistic without ever hearing it named. The leaflet is arguing that a portion of that grief was preventable, and that preventable grief is worth fighting.

Why this belongs in the church

The church is, for a great many Nigerians, the most trusted institution they belong to and the one that gathers them most regularly. If accurate health teaching is spoken there — clearly, without superstition, without turning the pulpit into a clinic — it reaches homes that no health campaign will ever reach. This is not a distraction from the church’s mission. It is an application of it.

Beloved, I wish above all things that thou mayest prosper and be in health, even as thy soul prospereth.

John's greeting holds body and soul together in one sentence. He does not set physical health against spiritual health; he wishes both, and he uses the state of the soul as the measure for the state of the body. A church that prays this verse over its members and then never speaks a word about how they live in their bodies has only prayed half of it.

A national average is made of individual households. Start with yours.

For reflection

1. How many people known personally to you have died before the age of sixty in the last five years?
2. What would change in your congregation if health were taught as part of loving one's neighbour?
3. What could your household do this month that, if every household did it, would raise a national average?

As Powerful as The Purpose Driven Life

FROM THE ORIGINAL LEAFLET

This book is as powerful as *The Purpose Driven Life* by Rick Warren, with over sixty million copies sold worldwide. I have prayerfully read PDL over ten times, each time gaining a new perspective.

The closing lines of the leaflet do something unusual. They compare a health book to a devotional book, and in doing so they tell you more about the person recommending it than about either volume.

What the comparison is actually claiming

It is not claiming that the two books teach the same things. *The Purpose Driven Life* is about why a person exists; *Prescription For Life* is about how a body lasts. The claim is about **power** — about a book's capacity to change how somebody lives after they have closed it. Measured that way, the comparison is a serious compliment, because most books change nothing at all.

The mention of sixty million copies is offered as evidence of that power. It is worth noticing, though, that sales measure reach rather than worth. A book can sell enormously and help little. The stronger evidence in this postscript is not the sales figure but the sentence that follows it.

"I have prayerfully read PDL over ten times"

This is the most revealing line on the whole leaflet. It shows what kind of reader is speaking: someone who does not read a serious book once and move on, but returns, prays over it, and expects to find something new because they themselves have changed in the interval. That is an old Christian discipline,

and it is nearly extinct in an age of scrolling.

It also quietly sets the standard for how *Prescription For Life* should be received. The recommendation is not *glance at this*. It is coming from a man who reads ten times, and it carries the implication that this book too deserves more than one hurried pass.

Reading prayerfully

- **Read slowly enough to be interrupted.** If a sentence convicts you, stop there rather than pressing on to finish the chapter.
- **Read with a pen.** Mark what you disagree with as well as what you accept; you will want both when you return.
- **Read with the Bible beside it.** No health book is Scripture. Let the greater book govern the lesser.
- **Read again later.** A book read at thirty and read again at fifty is effectively two different books, because the reader has changed.
- **Read to do.** End each session by naming one thing you will act on before the next.

Thy word have I hid in mine heart, that I might not sin against thee.

Psalm 119:11 (KJV)

The psalmist's method is not reading for information but hiding in the heart — the slow, repeated absorption of something until it governs behaviour without needing to be looked up. That method belongs first and supremely to Scripture. Applied in a lesser way to a useful book, it is exactly what reading a thing ten times means.

For reflection

1. Name a book, besides the Bible, that actually changed how you live. What made it different?
2. When did you last read anything twice?
3. What would it look like, practically, for you to read something prayerfully this week?

The Body You Were Given

Everything in this study edition has been practical: food, movement, blood pressure, risk. This section steps back and asks the question underneath all of it. Why should a Christian care about the body at all, when the soul is eternal and the body is not?

The body is not a temporary inconvenience

Some religious thinking has treated the body as a prison the soul is waiting to escape. Scripture does not. God formed the body with his own hands in the beginning, pronounced the whole creation good, became flesh himself in the person of his Son, raised that body from the dead, and promises resurrection rather than mere survival. The Christian hope is not escape from the body but its redemption.

What? know ye not that your body is the temple of the Holy Ghost which is in you, which ye have of God, and ye are not your own? For ye are bought with a price: therefore glorify God in your body, and in your spirit, which are God's.

1 Corinthians 6:19-20 (KJV)

Two arguments sit in that passage, and both bear directly on health. The first is presence: the body is a temple because God dwells in it, and what houses God is not to be treated carelessly. The second is ownership: *ye are not your own*. A steward may be careless with his own property; he may not be careless with property entrusted to him. Paul's conclusion is not shame about the body but glory in it — *glorify God in your body*.

Stewardship, not vanity

This must be said clearly, because the surrounding culture is loud and pulls hard in the opposite direction. Caring for the body as a steward is not the same as worshipping appearance. The aim is capacity, not admiration; service, not display. A woman who walks each morning so that she can still teach children at seventy and a woman who starves herself for a photograph are not doing the same thing, even if the scale reports the same number.

Favour is deceitful, and beauty is vain: but a woman that feareth the LORD, she shall be praised.

Proverbs 31:30 (KJV)

Health is for service

The reason to want more years and stronger ones is not simply to have them. In a Christian frame, health is fuel. The years gained are years available to raise children, to carry a household, to teach, to give, to preach, to bear somebody else's burden. A body kept in working order is a body that can still be spent.

I beseech you therefore, brethren, by the mercies of God, that ye present your bodies a living sacrifice, holy, acceptable unto God, which is your reasonable service.

A living sacrifice is offered while still alive, and it is offered bodily. Whatever else that means, it means the body is the instrument of worship and not merely its container.

A warning against a false gospel

Two errors wait at either side of this road. The first is neglect — treating the body as unspiritual and its care as worldly. The second is the error of supposing that careful living guarantees health, so that sickness must always be somebody's fault. It is not so. Good people fall ill. Careful people are struck down. Timothy had a stomach complaint and Paul prescribed for it rather than rebuking him. Trophimus was left at Miletum sick (2 Timothy 4:20). Job's friends built a whole theology on the assumption that suffering proves wrongdoing, and God told them plainly that they had not spoken rightly.

Care for the body as a steward. Do not judge the sick as though illness were a verdict.

For reflection

1. Do you think of your body as yours, or as entrusted to you? How would each answer change your habits?
2. Is there any way in which you have treated care of the body as unspiritual?
3. What service would you like to still be able to render at seventy or eighty?

Reading the Book Well: A Plan

A recommendation that produces nothing is a wasted page. This section turns the leaflet into a plan, so that the reader who agrees with it has something to actually do this week rather than a warm feeling to carry away.

Before the book arrives

1. **Write down your starting point.** Note your weight if you have a scale, your blood pressure if you know it, roughly how many minutes you move each day, and what you typically eat and drink between meals. You cannot measure a change from a starting point you never recorded.
2. **Get the basic checks done.** Blood pressure and blood sugar, at minimum. Do this whether or not you ever obtain the book.
3. **Name one person** who will ask you about this in a month's time, and tell them you have named them.

While reading

1. **Read one chapter at a sitting**, and stop to write the single sentence that struck you hardest.
2. **Keep a page for disagreements.** Where a claim surprises you, note it and raise it with a doctor rather than either swallowing or dismissing it.
3. **Do not wait until the end to begin.** Start the first change you are convinced of on the day you are convinced of it.
4. **Read it with someone**, ideally your spouse, since almost every change in diet happens or fails at a shared table.

After reading

1. **Choose three changes, not thirty**, and write them where you will see them.
2. **Set a review date** three months out, and repeat the measurements you took at the start.
3. **Pass it on.** Lend the book, or tell somebody plainly what you learned. Knowledge kept private helps one household; knowledge shared raises an average.

For the household

Most of what determines a family's health is decided by two or three people: whoever buys the food, whoever cooks it, and whoever sets the pattern of the evening. Changes made at that level affect everybody in the house without anybody having to be lectured. If you are one of those people, you hold more influence over the health of your family than any book does.

- Decide what is normally kept in the house, because what is in the house gets eaten.
- Decide what is drunk with meals.
- Decide whether the evening ends seated or on a walk.
- Decide whether the family eats together, since a shared table slows eating and opens conversation.

Go thy way, eat thy bread with joy, and drink thy wine with a merry heart; for God now accepteth thy works.

Ecclesiastes 9:7 (KJV)

Let this verse guard the whole plan against grimness. The goal is not a joyless table policed by rules, but a household that eats with gladness and lives long enough to keep doing so.

For reflection

1. Which of the steps above can you do this week without spending money?
2. Who in your household makes the decisions listed here, and have you ever discussed them together?
3. What will you measure, and when will you measure it again?

A Prayer and a Commitment

This booklet began with a single page recommending a book and has ended with a plan for a household. What remains is to bring the whole matter before the Lord, since no diet survives on resolve alone and no change of life is sustained by information.

A prayer

Almighty God, maker of my body and giver of my breath, I thank you for the frame you formed and the days you have appointed. Forgive me where I have treated your gift carelessly, excused what I knew to be harmful, or called neglect of my body a spiritual matter when it was only laziness. Forgive me too where I have made food my comfort and my master.

Teach me to eat and to move as one who has been entrusted with something that is not his own. Give me the humility to seek help where I need it, the discipline to continue when nothing appears to change, and the grace to begin again the day after I fail. Keep me from vanity, from fear, and from judging those who are sick.

Grant health to my household, and prolong our days for your service and not for our own ease. Bless our nation; raise up doctors, teachers and pastors who will speak truth about these things; and may the years you give us be spent in loving you and loving our neighbour. In the name of Jesus Christ, who took a body and bore it to the cross for us. Amen.

A commitment

If you are ready to act rather than only to agree, put your hand to these and name a date:

1. I will have my blood pressure and blood sugar checked by _____.
2. I will make these three changes to how I eat and move: _____,
_____, _____.
3. I will move my body deliberately for _____ minutes, _____ days each week.
4. I have told _____ about this, and they will ask me on _____.
5. I will read *Prescription For Life* and, God willing, obtain a copy by _____.

Do not wait until you have the book to begin what you already know.

A final word

The leaflet that this edition is built on is one page long and makes a small, honest claim: that here is a book worth your time, and that reading it could add years to Nigerian lives. These notes have tried to

show why such a claim belongs on the shelf of a ministry about love, and why the body is a fit subject for Christian care. But a study edition, like a recommendation, cannot do the thing itself.

But be ye doers of the word, and not hearers only, deceiving your own selves.

James 1:22 (KJV)

Close the booklet. Take the walk. Make the appointment. Then find the book.